



Can non-pharmacological interventions prevent relapse in adults who have recovered from depression? A systematic review and meta-analysis of randomised controlled trials



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HIGHLIGHTS

- We evaluated non-pharmacological interventions given after recovery from depression.
- The main meta-analysis incorporated 27 two-way comparisons and 2742 participants.
- CBT, MBCT and IPT reduced risk of relapse 12 months after recovery.
- The efficacy of MBCT following acute psychotherapy was largely untested.
- Longer term outcomes and the impact of acute treatment need further exploration.

ARTICLE INFO

Article history:

Received 6 August 2014

Received in revised form 8 April 2015

Accepted 15 April 2015

Available online 20 April 2015

Keywords:

Depression

Relapse

Prevent

Psychological therapy

Long-term

ABSTRACT

Objective: To identify studies of non-pharmacological interventions provided following recovery from depression, and to evaluate their efficacy in preventing further episodes.

Method: We identified relevant randomised controlled trials from searching MEDLINE, Embase, PsycINFO, CENTRAL, and ProQuest, searching reference and citation lists, and contacting study authors. We conducted a meta-analysis of relapse outcomes.

Results: There were 29 eligible trials. 27 two-way comparisons including 2742 participants were included in the primary analysis. At 12 months cognitive-behavioural therapy (CBT), mindfulness-based cognitive therapy (MBCT), and interpersonal psychotherapy (IPT) were associated with a 22% reduction in relapse compared with controls (95% CI 15% to 29%). The effect was maintained at 24 months for CBT, but not for IPT despite ongoing sessions. There were no 24-month MBCT data. A key area of heterogeneity differentiating these groups was prior acute treatment. Other psychological therapies and service-level programmes varied in efficacy.

Conclusion and implications: Psychological interventions may prolong the recovery a person has achieved through use of medication or acute psychological therapy. Although there was evidence that MBCT is effective, it was largely tested following medication, so its efficacy following psychological interventions is less clear. IPT was only tested following acute IPT. Further exploration of sequencing of interventions is needed.

Systematic review registration number: PROSPERO 2011:CRD42011001646

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